



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : عزه ابراهيم عبده محمد
المحافظة : الجيزة الحي / المدينة : الجيزة
رقم البطاقة : 27608271900244 رقم التليفون : 01116654583
العمر (تاريخ الميلاد) : 27-08-1976 المهنة : ربه منزل
العنوان : =
تاريخ تسجيل الإستمارة : 07-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : -----
الاسم : د/ندى حسن الوظيفة : -----
الاسم : د/نرمين رمضان الوظيفة : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : قسم : -----
جهة العلاج : نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: عزه ابراهيم عبده محمد National ID: 27608271900244	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



عزہ

ابراهيم عبده محمد

٢٧ ش الشهيد-سعد زغلول

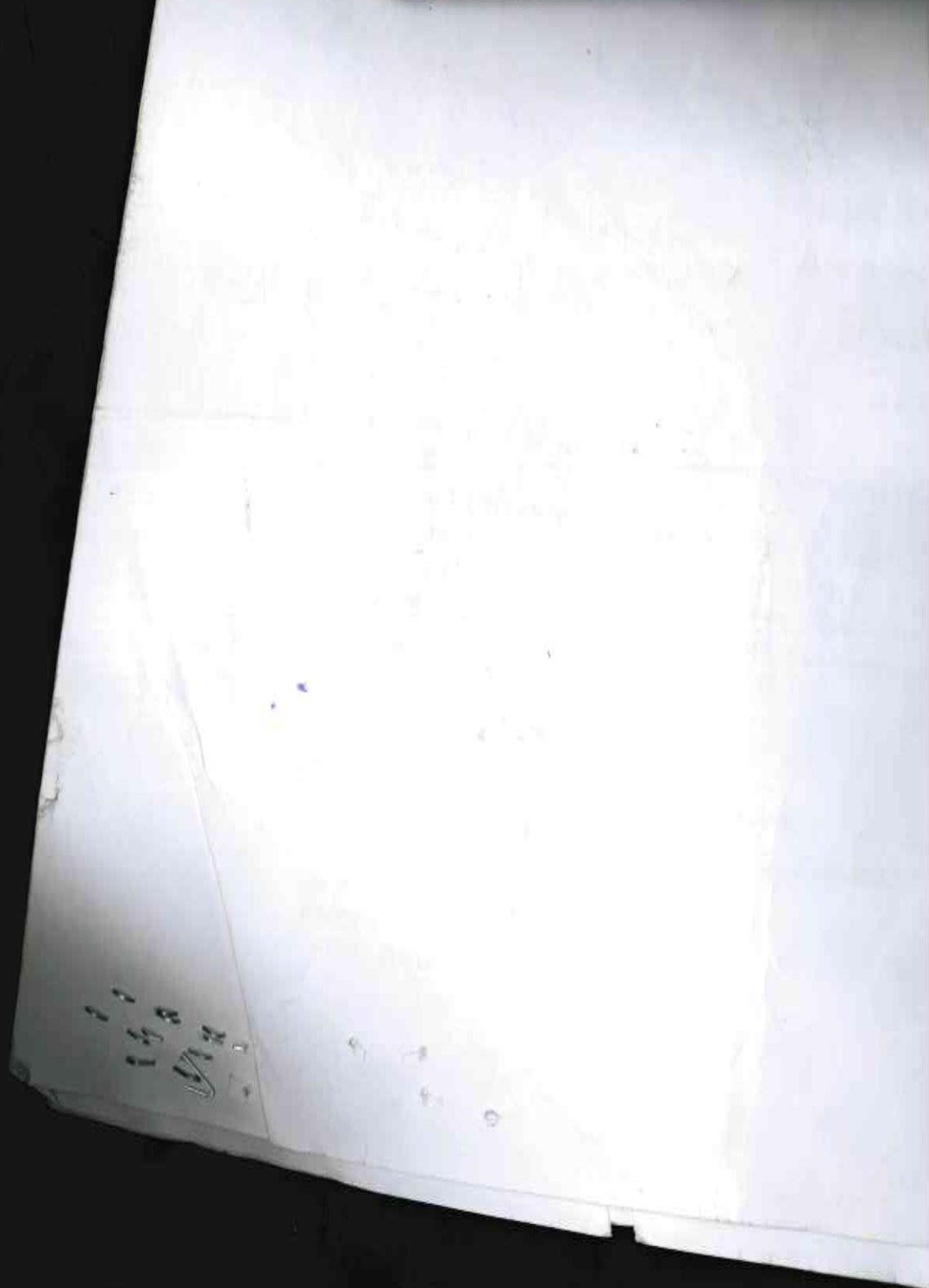
الجيزة - الجيزة



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JY4411438

national identity





وزارة الصحة
مملكة العربية السعودية

الهيئة العامة
للصحة العامة

MDT FORM

15 / 9 / 2026

G

Name	سجيرة المريضة	MRN	
Age	49	ID	
Menopausal status	perimenopausal	Phone	0116955175
NRS SCORE		marital status	divorced + 3 offspr → 15 yrs
BMI		Family history	(-)
Co morbidity	H: HTN: Concor / Targ: hypothyroidism on thyrocare 800 S: Echo:	REASON OF MDT	
Tensitometry 11 years old Clinical Data			

Bx: INC Grade III Herenriched. 1 year ago.

ER: (-) PR: (-) HER 2: Enriched Ki67: 25% +3

Sonomammography (7/2/2026)

L Breast: uod. 3 o'clock 15x0.8 cm + area of altered echogenicity → 4x2 cm

6 mm. from skin, 8 cm from nipple

Mets Workup: 1 o'clock para areolar altered echogenicity 9x5 mm

6 o'clock 4x11 (gst)

L ALN: level 4 mm critical thickening

(Exam 9-7-2026)

Recommendations:

L Breast: uod area of asymmetry: 12.7x6.3 cm, 1.5 cm from nipple

Completely u.s: L uod

ALN: level 1 single LN 35 cortical → Indeterminate

CRAB: Chest: level 1 axilla ~ 1.3x1.8 cm

L Supradar → 0.7x0.8 cm

Abdomen: free: Caldar gall bladder

MDT OUTCOME & PLAN

→ Lap inspection & Target therapy? + Chemotherapy

→ Targeted Herceptin post 2 weeks

to do internal.

Signature: 2/9/26 2/10/26

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	psychiatric

Patient name: AZA EBRAHEM ABDO MOHAMED	Age:
Modality Name: CT	Study Date: 8/29/2026 10:54:55 AM
Patient id: 100422179	

THORACIC IMAGING UNIT

POST CONTRAST CT SCAN OF THE CHEST REVEALED:-

****Given history of left breast invasive duct carcinoma, for metastatic work up.**

- Left breast outer quadrant heterogeneously enhancing parenchyma mounting to formation to irregular mass lesion associated with mild skin thickening.
- Pathologically enlarged left axillary lymph nodes, the largest measures about 1.3 x 1.8 cm along its maximum axial dimensions.
- Prominent left supraclavicular lymph nodes, the largest measures about 0.7 x 0.8 cm along its maximum axial dimensions.
- Right middle lobe tiny pulmonary nodule measuring about 2mm
- No evidence of masses, consolidative patches or cavitations.
- Middle lobar and lingular parenchymal bands.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No gross cardiomegaly. Minimal pericardial effusion.
- No pleural sac collections.
- Aortic atheromatous calcifications.
- Enlarged thyroid gland showing inhomogeneous texture, for ultrasound correlation.
- Scanned bone window shows dorsal spondylosis.

OPINION:-

- Left breast malignant process with ipsilateral pathological axillary lymphadenopathy, as described.
- Right middle lobe tiny pulmonary nodule for follow up.

Dr. Nouran M. Abdallah, MSc

Dr. Nourhan Mohamed, MD

Cairo university – faculty of medicine
Cairo university hospital
Radiology department



جامعة القاهرة - كلية الطب
مستشفيات جامعة القاهرة
قسم الأشعة التشخيصية

Patient Name: AZA EBRAHEM ABDO MOHAMED	Date: 29/08/2026
Age:	ID: 100422179

Multislice post contrast CT scan of the abdomen and pelvis with multiplanar reformatted images revealed:

****Given history of breast invasive ductal carcinoma.**

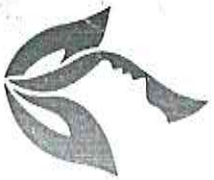
****No previous studies available for comparison.**

- Mildly enlarged liver showing homogenous parenchymal pattern. No evidence of sizable enhancing hepatic focal lesions or biliary radicles dilatation.
- The gall bladder is seen containing a radio dense stone seen measuring about 7 mm with no evidence of pericholecystic fluid collection or mural thickening.
- Normal CT appearance of the spleen (Few small splenules are noted), pancreas, suprarenal glands, aorta, IVC and pelvic organs.
- Normal size and excretory capacity of both kidneys.
- bulky uterus with lobulated outlines, could be due to fibroid, for sonographic correlation.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

OPINION:

- **No evidence of sizable metastatic lesions.**
- **Mild hepatomegaly.**
- **Calcular gall bladder.**

Dr, Marina Ishaq, MSc
Dr.Rasha Mostafa,MD



Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
مستشفيات جامعة القاهرة

PATHOLOGY REPORT



Name: عزه ابراهيم عبده محمد

Patient's I.D.: 27608271900244

Age: 49 Y.

Pathology Number: 220/2026

Specimen Received Data: 28 / 07 /2026

Date Report Issued: 09/ 08 /2026

Clinical data: Left breast UOQ mass, BIRADS 5.

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of breast tissue showed an invasive malignant tumor formed of groups, micropapillae and few tubules of moderately to markedly anaplastic epithelial cells. Surrounding desmoplastic stroma with 5-10% TILs was seen. An associated high grade DCIS component with comedo necrosis was also detected.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed weak positive nuclear staining in <1% of tumor cells by chromogen (Score 2/8).
Examination of section treated for progesterone receptors revealed weak positive nuclear staining in <1% of tumor cells by chromogen (Score 2/8).
Examination of section treated for Her-2/neu revealed strong complete membrane staining in most of tumor cells by chromogen (score 3+).
Examination of section treated for Ki67 revealed positive nuclear stain in average of $\pm 25\%$ of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

Invasive Duct Carcinoma, NST, Grade III.

- Negative For Estrogen Receptors.
- Negative For Progesterone Receptors.
- Positive For Her-2/neu Expression (Score 3+).
- $\pm 25\%$ Ki67 Proliferative index.

PROF. DR. PASSANT ESSAM SHIBEL
DR. SARA ABOU-FANDOUD

Passant
S. Abo- Fandoud

CAIRO UNIVERSITY HOSPITALS
FACULTY OF MEDICINE
RADIO DIAGNOSIS DEPARTMENT

مستشفيات جامعة القاهرة
كلية الطب
قسم الأشعة التشخيصية

WOMEN IMAGING DEPARTMENT

Patient Name: Azza Ibraheem Abdo
Ref. Department:

Date: 09-07-2026

Contrast enhanced mammography

Indication :

- Left breast palpable lump.
- Her sonomammography revealed: left breast UOQ focal asymmetry with overlying suspicious segmental micro-calcification , **complementary US** revealed corresponding left UOQ palpable area of parenchymal alteration harboring few hypoechoic ill defined ante-parallel lesions with suspicious microcalcifications and ipsilateral indeterminate LN (BIRADS 4).

Findings:

- Bilateral scattered fibroglandular breast parenchyma (ACR b)
- Bilateral minimal background enhancement.

Left breast:

- Left breast UOQ area of asymmetrical segmental non mass enhancement seen measuring about 12.7x 6.3 cm., extending about 1.5 cm from the nipple and about 3 cm from the underlying chest wall, corresponding to the previously reported left UOQ focal asymmetry with overlying suspicious segmental micro-calcification.
- Left breast LOQ eggshell calcifications corresponding to the previously reported oil cyst.
- **Complementary US revealed:** corresponding left UOQ palpable parenchymal alteration harbouring few hypoechoic ill defined ante-parallel lesions with suspicious microcalcifications extending from 12 o'clock to 3 o'clock about 3 mm from the overlying skin.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- **Left axillary level 1 single LN** showing uniform cortical thickening reaching 3.5 mm yet with preserved central fatty hilum ; indeterminate LN.

Right breast:

- No pathological mass or non mass enhancement.
- No solid or cystic lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.

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WOMEN IMAGING DEPARTMENT

Patient Name: Azza Ibraheem Abdo
Ref. Department:
Date: 09 / 07/2026

Ultrasound guided biopsy of the Breast

- Multiple core biopsies from left UOQ palpable altered parenchyma and suspicious microcalcifications (BIRADS 4) for histopathological correlation.
- If positive ER, PR and HER-2 NEU

Thanks for reference
Dr. Neveen Gamal, MD

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WOMEN IMAGING DEPARTMENT

- Non specific axillary LNs.

Opinion:-

- Left breast UOQ asymmetrical pathological segmental non mass enhancement corresponding to left UOQ asymmetry , suspicious microcalcifications with underlying palpable altered parenchyma as described (BIRADS 4) for histopathological correlation.
- No right breast enhancing lesions (BIRADS 2).

Thanks for referral,
Dr.Neveen Gamal , MD

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RADIO DIAGNOSIS DEPARTMENT

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WOMEN IMAGING DEPARTMENT

Patient Name: Azza Ibrahim Abdo

Date: 07/02/2026

Bilateral Mammography with High resolution Ultrasound
Examination

Indication: Left breast lump

In comparison to the last ultrasound dated 02/07/2025 and the last sonomammography dated 20/10/2024 the current study revealed:

Findings:

- Scattered fibro glandular breast parenchyma (ACR B)
- Left breast UOO area of focal asymmetry with suspicious segmental microcalcifications, Complementary US shows 3 O'clock zone irregular shaped lesion with indistinct margins measuring 15x8mm (surrounded by area of altered echogenicity measuring 4x2 cm, microcalcific foci and few hypoechoic nodules. The previously mentioned mass lesion is 6 mm from the skin and 8 cm from the nipple.
- Left breast 12 o'clock para areolar area of altered echogenicity and cluster microcysts measuring about 9x5mm. ↓
- Left breast 6 o'clock rounded hypoechoic lesion with egg shell calcification likely oil cyst. ↓
- Right breast 12 o'clock IMLN. ↓
- Normal contour and skin thickening
- Normal nipple / areola complexes.
- Right nonspecific axillary lymph nodes with preserved fatty hilum.

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WOMEN IMAGING DEPARTMENT

- Left axillary level 1 enlarged lymph node, one of them show focal cortical thickening (4 mm)

Opinion :-

- Left breast UOQ mass lesion showing irregular margins, with surrounding segmental area of altered echogenicity, suspicious microcalcifications and few nodules as well as left axillary focal thickening (BIRADS 4 a) for contrast enhanced mammography and rebiopsy.
- Unremarkable sono mammographic study of right breast (BIRADS 2).

Thanks for referral,

Dr. Manar Osman, MD.

Dr. Mahmoud Mekhaimar, MD

Dr. Manar Magdy, M.Sc

جمهورية مصر العربية
بطاقة تحقيق الشخصية



عزہ
ابراهيم عبده محمد
٢٧ ش الشهداء-سعد زغلول
الجيزة - الجيزة

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